

Clinical Intelligence Trend Report — 30 days

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient ID	934297-0122 (example)	Monitoring period	2024-05-31 → 2024-06-30
Data resolution	Hourly aggregates (min/avg/max)	Window requested	30 days

Narrative Interpretation

Most frequent episodes: Very low heart rate (HR min <45) (n=6); Low heart rate (HR avg <50) (n=6); High respiratory rate (RR avg >24) (n=4). Episodes occurred in clustered bursts rather than isolated hours. Some windows show concurrent low HR and elevated RR, suggesting coupled physiologic stress rather than isolated variation. Data quality: 26 low-confidence hours (low sample-count). Verify extremes against measurement context. Trend assessment: Escalating instability toward end of monitoring.

High-Priority Episodic Events (Top 8)

Event type	Window	Hours	Key vitals	Comments
Very low heart rate (HR min <45)	06-23 21:00 → 06-24 06:00	9	Min HR 40 ; Peak HR(avg) 56; Peak RR(avg) 29	very low minima
High respiratory rate (RR avg >24)	06-24 00:00 → 06-24 10:00	10	Min HR 40 ; Peak HR(avg) 52; Peak RR(avg) 29	RR elevation
Low HR + High RR overlap	06-24 00:00 → 06-24 06:00	6	Min HR 40 ; Peak HR(avg) 49; Peak RR(avg) 29	HR–RR overlap
Very low heart rate (HR min <45)	06-24 07:00 → 06-24 11:00	4	Min HR 42 ; Peak HR(avg) 49; Peak RR(avg) 28	very low minima
Very low heart rate (HR min <45)	06-25 01:00 → 06-25 06:00	5	Min HR 40 ; Peak HR(avg) 51; Peak RR(avg) 15	very low minima
Low heart rate (HR avg <50)	06-23 22:00 → 06-24 06:00	8	Min HR 40 ; Peak HR(avg) 49; Peak RR(avg) 29	review context
High heart rate (HR avg >100)	06-27 20:00 → 06-28 03:00	7	Min HR 91 ; Peak HR(avg) 104; Peak RR(avg) 12	HR elevation
Low HR + High RR overlap	06-24 07:00 → 06-24 10:00	3	Min HR 42 ; Peak HR(avg) 49; Peak RR(avg) 28	HR–RR overlap

Statistical Summary + Visual Context

Metric	HR avg (bpm)	RR avg (brpm)
Mean	64.0	18.6
P5 / P95	52.1 / 93.7	11.0 / 22.6
Min / Max	41.3 / 103.6	9.7 / 29.1
Low-confidence hours (cnt<30)	26	



